...ARTMENT OF LABORATORY SERVICES

THE MISSION HOSPITAL DURGAPUR | FUTURISTIC TECHNOLOGY. BRILLIANT MINDS.

Patient Name	Mr. ALOKE KUMAR MUKHERJEE	Bill No	OPCA326/173197
Source	OPD	Sample Date	03/09/2026 3:22PM
UHID	842747	Receiving Date	03/09/2026 4:24PM
Age/Gender	66 Yrs/Male	Report Date	04/09/2026 1:23AM
Bed No/Ward	OPD	Report Status	Final
Referred By	Dr. ASHISH KRISHNA	Lab No	362504047089
Sample Name	Serum	ManualDept No.	

HIV 1 & 2 ANTIBODY (ECI)

HIV 1 / 2 ANTIBODY & HIV P24 ANTIGEN (ECI)	0.12	Index	0 - 1
INTERPRETATION-	Non Reactive		

Interpretation:

Reference Group	Value
Non Reactive	<0.90
Borderline	0.90-1.00
Reactive	>1.00

Acquired Immunodeficiency Syndrome (AIDS) is caused by at least two types of Human Immunodeficiency Viruses designated HIV-1 and HIV-2. The test is for the simultaneous qualitative detection of antibodies to Human Immunodeficiency Virus types 1, including group M and O, and/or 2 (anti-HIV-1 and anti-HIV-2) and HIV p24 antigen in human serum and plasma. Detection of HIV p24 antigen that may be present in early seroconversion before the onset of antibody response, helps in earlier diagnosis of HIV infection. If test result is >= 1.00, the sample should be considered Reactive and tested by supplemental tests to confirm the result. In the case of repeatedly " Borderline" results, analysis of follow-up samples is recommended. This is a screening test. A negative result implies that no anti HIV 1 or anti HIV 2 antibodies or p24 antigen have been detected in sample by this method. However, it does not rule out possibility of exposure to or infection with HIV 1 or 2. This could also mean that either patient has not been exposed to HIV 1 & 2 or sample has been tested during "WINDOW PHASE" (Before development of detectable antigen and antibodies). Therefore, for a subject suspected of HIV exposure, it is advisable to repeat the test after 2 weeks to 3 months. **NACO guidelines are followed for testing of samples. All reactive samples should be confirmed by using HIV Western Blot/ PCR.**

----------**End Of Report**----------

Dr. PALASH DAS
DPH,MD(MICROBIOLOGY) Regd. No. 64476(WBMC)
CONSULTANT (MICROBIOLOGY INFECTION CONTROL)

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Printed at :21/08/2026 12:00

THE MISSION HOSPITAL DURGAPUR | FUTURISTIC TECHNOLOGY BRILLIANT MINDS.

DEPARTMENT OF CARDIOLOGY/CARDIOLOGY INVESTIGATION

952504086151/ Final

Patient Name :	Mr. ALOKE KUMAR MUKHERJEE	Report No :	952504086151
UHID / IP NO	842747 (262305)	Delivery Date:	03/09/2026 4:20PM
Age/Gender :	66 Y/M	Invoice Date :	03/09/2026 3:22PM
Contact No :	8583042932	Invoice No :	OPCA326/173197
Referred By :	Dr.ASHISH KRISHNA MBBS, MS, FMAS		

ECHO DOPPLER

M-mode Measurements Values

Aorta	33mm	LV ed	43mm
LA	36mm	LV es	25mm
LVEF	60%	IVS ed	13mm
		PW (LV)	12mm

FINAL IMPRESSION

- Concentric left ventricular hypertrophy.
- Normal LV systolic function.

DR NADEEM AFROZ
MD DM(CARD)
(CONSULTANT INTERVENTIONAL CARDIOLOGIST)

DR DEBARGHA DHUA
MD DM(CARD)

DR TAPAN KR MATIA
DNB(CARD) FESC

DR SUBHABR
MD DM(CARI

* Suggested clinical co-relation
* This report is not for medico legal purposes

Prepared By: M PAL

Dr. ABHIJIT GHOSH
MD, PGDCC
FELLOWSHIP IN ECHOCARDIOGRAPHY

(ASSOCIATE CONSULTANT)

DEPARTMENT OF CARDIOLOGY/CARDIOLOGY INVESTIGATION

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ECHO DOPPLER

01. Cardiac Chambers:
Concentric left ventricular hypertrophy. Other all Cardiac chambers are normal.

02. Valve morphology and function :
Mitral Valve : Morphology : Normal Doppler : E>A
Regurgitation: Absent

Tricuspid Valve : Morphology : Normal
Regurgitation: Absent

Aortic Valve :Morphology : Trileaflet, PSG 7 mmHg
Regurgitation: Absent

Pulmonary Valve: Morphology: Normal
Regurgitation: Absent

03. Regional wall motion abnormality:
No regional wall motion abnormality present.

04. Ventricular systolic and diastolic function:
Normal biventricular systolic function.
Normal LV diastolic function.

05. Great arteries:
Normally related great arteries Normal in size.

06. Inter-atrial and Inter- ventricular septum:
Intact Inter-atrial and Inter- ventricular septum.

07. Pulmonary and systemic venous drainage:
Normal Pulmonary and systemic venous drainage.

08. Pericardium/Others:
No thickening, or effusion.
IVC normal.

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EOSINOPHILS (VCS/Leishman staining and microscopy)	0.4	L	%	1 - 5
BASOPHILS (VCS/Leishman staining and microscopy)	0.7		%	0 - 2
Other cells				
RBC INDICES				
MEAN CORPUSCULAR VOLUME (MCV) (Derived From RBC Histogram)	76.10	L	fl	83.00 - 101.00
MCH (CALCULATED)	22.0	L	pg	27.0 - 32.0
MCHC (CALCULATED)	28.9	L	g/dl	31.5 - 34.5
RDW-CV (Derived From RBC Histogram)	20.6	H	%	11.0 - 15.0

Bijan Basak

Dr. BIJAN BASAK
MD (Pathology), PDF (Oncopathology)
Consultant Oncopathologist | REGD. NO: (WBMC65329)

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Referred By	Dr. ASHISH KRISHNA	Lab No	132504156353
Sample Name	W. B. EDTA	ManualDept No.	

Investigation Name	Result	Units	Bio. Ref.Interval

BIOCHEMISTRY

GLYCOSYLATED HAEMOGLOBIN, HBAIC

HBA1C - GLYCOSYLATED HEMOGLOBIN* (HPLC)	4.7	%	* Non Diabetic : < 5.7; * Pre Diabetic : 5.7- 6.4; *Diabetic : > or = 6.5; *Action suggested : > 8.0

REMARKS:

1. HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
2. HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
3. Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapsone, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
4. HbA1c may be increased in patients with polycythemia or post-splenectomy.
5. Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.

Dr. MD RASHID AHSAN LODHI
MBBS, MD, 67603 (WBMC)
Consultant Biochemistry

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HAEMATOLOGY

BT & CT

BLEEDING TIME	02 min 30 sec		
CLOTTING TIME (CT)	05 min 15 sec		

Bijan Basak

Dr. BIJAN BASAK
MD (Pathology), PDF (Oncopathology)
Consultant Oncopathologist | REGD. NO: (WBMC65329)

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...ge/Gender	66 Yrs/Male	Report Date	03/09/2026 5:17PM
...ed No/Ward	OPD	Report Status	Final
...eferred By	Dr. ASHISH KRISHNA	Lab No	322504073901
...mple Name	Serum	ManualDept No.	

Investigation Name	Result	Units	Bio. Ref.Interval

SPECIAL BIOCHEMISTRY

...YROID STIMULATING HORMONE (TSH)

...H-THYROID STIMULATING HORMONE TRA (CLIA, VITROS 5600)	3.18	µIU/mL	**Premature** 28-36 wk : 0.7- 27.0 µIU/mL. ; **Cord blood**(>37 wk): 2.3 - 13.2 µIU/mL; **Children:** Birth-4 days: 1.0-39.0 µIU/mL; 2 - 20 wk: 0.7- 64.0 µIU/mL **Adults:** 21- 54 yr: 0.4 - 4.2 µIU/mL; 55 - 87 yr: 0.5 - 8.9 µIU/mL; **Pregnancy:** First trimester: 0.3 - 4.5 µIU/mL; Second trimester: 0.5 - 4.6 µIU/mL; Third trimester: 0.8 - 5.2 µIU/mL

----------**End Of Report**----------

Dr. MD RASHID AHSAN LODHI
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Consultant Biochemistry

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Sample Name	W. B. EDTA	ManualDept No.	

ALKALINE PHOSPHATASE/ALP (Multi-point rate - PMPP, AMP Buffer (VITROS 5600))	152	H	U/L	53 - 128
TOTAL PROTEIN (Colorimetric- Biuret,end point (VITROS 5600))	7.9		g/dl	6.3 - 8.2
ALBUMIN (Colorimetric - Bromcresol Green (VITROS 5600))	3.7		g/dl	3.5 - 5.0
GLOBULIN (CALCULATED)	4.2	H	g/dl	2.3 - 3.5
A/G RATIO (ALBUMIN/GLOBULIN RATIO)	0.88		Ratio	1.1 - 2.5

***DELTA BILIRUBIN:**

Delta bilirubin is conjugated biliribin that is covalently bound to albumin, a protein in the blood. It is marker of impaired hepatic excretion of conjugated bilirubin and can indicate the effectiveness of biliary drainage especially in obstructive jaundice .

HOW*DELTA BILIRUBIN FORMS:

This binding occurs during prolonged cholestasis (impaired bile flow) or when the liver's ability to excrete conjugated bilirubin is compromised.

***DELTA BILIRUBIN(DELB)= TBIL – (Bu + Bc),** is not expected in healthy adults or neonates. In untreated patients with liver pathology , it will be accompanied by **Bc** and **TBIL** elevations. If (Bu +Bc) is < TBIL result, and TBIL and Bc are normal , a positive calculated DELB value should not interpreted as an increase in Delta bilirubin

SIGNIFICANCE OF *DELTABILIRUBIN:

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RADIOLOGY INVESTIGATION DEPARTMENT

UHID / Bill No	842747 / OPCA326/173197	RIS No	992583685
Patient Name	Mr. ALOKE KUMAR MUKHERJEE	Age/Gender	66 Yrs/Male
Referred By	Dr. ASHISH KRISHNA	Bed No/Ward	OPD
Prescribed By	Dr. ASHISH KRISHNA	Receiving Date	03/09/2026 3:24PM
Bill Date	03/09/2026 3:22PM	Report Date	04/09/2026 3:03PM
Company	CASH	Report Status	Final

X-RAY: CHEST (PA)

Observations:

- No parenchymal lesion is seen in the lung fields.
- Both hila are normal.
- Heart shadow is within normal limits.
- Contour of the diaphragm is within normal limits.
- Costophrenic angles are clear.
- Bony cage is normal.

Dr. ARNAB MANDAL
MBBS,MD,DNB FMF (UK) ,CERTIFIED
ID 315012 REGD .NO : 92078(WBMC)
ASSOCIATE CONSULTANT

Prepared By: 166 Printed By: D248 Printed At: 04/09/2026 03:03 PM

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UREA AND CREATININE

BLOOD UREA (ENZYMATIC IFCC)	58	H	mg/dL	19 - 43
CREATININE (ENZYMATIC IFCC)	1.4	H	mg/dL	0.7 - 1.3

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LIVER PROFILE /LFT. (t.bil, sgot, sgpt, alkp,t.prot, alb, a/g ratio)

BILIRUBIN TOTAL (Colorimetric - Diphylline, Diazonium (VITROS 5600))	1.8		mg/dL	ADULT: 0.3 - 1.3 mg/dL
BILIRUBIN UNCONJUGATED (BU) (Colorimetric - Diphylline, Diazonium (VITROS 5600))	1.3	H	mg/dL	0.0 - 1.1
BILIRUBIN CONJUGATED (BC) (Colorimetric - Spectrophotometric)	0.2		mg/dL	0.00 - 0.30
DELTA BILIRUBIN (CALCULATED)	0.3		mg/dL	0.0-0.2 mg/dl
AST/SGOT (Multi-point Rate (VITROS 5600))	48		U/L	FEMALE: 14 - 36 U/L, MALE: 17 - 59 U/L
ALT/ SGPT (Multi-point rate - UV with P5P (VITROS 5600))	16		U/L	Female: <35 U/L Male: <50 U/L
GGT (Multi-point-G-glutamyl-p-nitroanilide (VITROS 5600)	124	H	U/L	15 - 73

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Biliary drainage: An increase in the percentage ofserum delta-bilirubin can indicate effective biliary drainage

Cholestasis: Delta bilirubin levels can help interpret the degree of cholestasis (a condition where bile flow is blocked) in case of hepatobiliary obstruction.

Prognosis: Delta bilirubin can help predict the disease chronicity and may be a useful marker of liver graft rejection and acceptance.

Inverse Relationship: There's an inverse relationship between delta bilirubin and serum albumin levels, meaning that as delta bilirubin increase, albumin levels tend to decrease.

Not Directly Measured: While delta bilirubin is clinically relevant, it cannot be measured directly in real time for diagnosis proposes in clinical laboratories.

CLINICAL APPLICATIONSOF *DELTA BILIRUBIN:

- **Obstructive Jaundice:** Delta bilirubin analysis can help distinguish between well-drained and poorly-drained patients with obstructive jaundice.
- **Liver Disease:** it can be useful in diagnosing and treating various liver diseases, including cholestasis and liver graft rejection.
- **Pediatric Jaundice:** Delta bilirubin can be used in the evolution of pediatric jaundice conditions like biliary atresia

IMPORTANT NOTE: Further large- scale studies are needed to fully understand the clinical usefulness of delta bilirubin

----------**End Of Report**----------

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CBC (HB.TC.DC.RBC.PCV.MCV.MCHC.RDW.PLATELET)

HEMOGLOBIN (Photometric Measurement)	10.9	L	g/dl	13.0 - 17.0
TOTAL LEUKOCYTE COUNT (TLC) (Coulter Principle)	11800.00	H	cells/cumm	4000.00 - 11000.00
RBC COUNT (RED BLOOD CELL) (Coulter Principle)	4.98		Million/µL	4.50 - 5.50
PCV (HAEMATOCRIT) (CALCULATED)	37.9	L	%	40.0 - 50.0
PLATELET COUNT (Coulter Principle/Neubauer Chmaber Microscopy)	1.38	L	lakh/cumm	1.50 - 4.00
DIFFERENTIAL COUNT				
NEUTROPHILS (VCS/Leishman staining and microscopy)	79.9	H	%	50 - 70
LYMPHOCYTES (VCS/Leishman staining and microscopy)	14.8	L	%	25 - 30
MONOCYTES (VCS/Leishman staining and microscopy)	4.2	L	%	5 - 10

Dr. BIJAN BASAK
MD (Pathology), PDF (Oncopathology)
Consultant Oncopathologist | REGD. NO: (WBMC65329)

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PLASMA GLUCOSE R

PLASMA GLUCOSE RANDOM (Colorimetric - GOD POD (VITROS 5600))	106	mg/dL	74 - 140

Dr. MD RASHID AHSAN LODHI
MBBS, MD, 67603 (WBMC)
Consultant Biochemistry

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A Unit of Durgapur Medical Centre Pvt. Ltd. CIN: U85110WB1987PTC042580

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& by limits of simplicity and specificity of the assay procedure.

THE MISSION HOSPITAL DURGAPUR | FUTURISTIC TECHNOLOGY. BRILLIANT MINDS.

EPARTMENT OF LABORATORY SERVICES

Patient Name	Mr. ALOKE KUMAR MUKHERJEE	**Bill No**	OPCA326/173197
Source	OPD	**Sample Date**	03/09/2026 3:22PM
UHID	842747	**Receiving Date**	03/09/2026 4:16PM
Age/Gender	66 Yrs/Male	**Report Date**	03/09/2026 6:38PM
Bed No/Ward	OPD	**Report Status**	Final
Referred By	Dr. ASHISH KRISHNA	**Lab No**	132504156353
Sample Name	W. B. EDTA	**ManualDept No.**	

6. Trends in HbA1c are a better indicator of diabetic control than a solitary test.

7. Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.

8. HbA1c target in pregnancy is to attain level <6 % .

9. HbA1c target in pediatric age group is to attain level < 7.5 %.

10. The presence of some hemoglobinopathies/hemoglobin variants may be missed by some platforms used for measuring HbA1c values. in these cases, the HbA1c result may vary or give deviant results, depending on the platform used. Alternative measures of glycemic control like fructosamine or glycated albumin may be considered in such cases.

11. In presence of hemoglobinopathy/hemoglobin variants in homozygous/compound heterozygous conditions alternative forms of testing such as fructosamine or glycated albumin may be considered.

Method : Ion-exchange high-performance liquid chromatography (HPLC).

Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023

Dr. MD RASHID AHSAN LODHI
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THE MISSION HOSPITAL DURGAPUR | FUTURISTIC TECHNOLOGY. BRILLIANT MINDS.

Patient Name	Mr. ALOKE KUMAR MUKHERJEE	Bill No	OPCA326/173197
Source	OPD	Sample Date	03/09/2026 3:22PM
UHID	842747	Receiving Date	03/09/2026 3:36PM
Age/Gender	66 Yrs/Male	Report Date	03/09/2026 8:37PM
Bed No/Ward	OPD	Report Status	Final
Referred By	Dr. ASHISH KRISHNA	Lab No	925040308763
Sample Name		ManualDept No.	

PROTHROMBIN TIME(PT) & INR

PROTHROMBIN TIME (PT WITH INR)*
(Optical Nephelometry)

TEST VALUE	**15.9**	H	Seconds	10.85 - 12.89
CONTROL	12.0		Seconds	
INR (CALCULATED)	1.34			
ISI	1.04			

ISI Value : The lot specific and technique -specific (mechanical and photo-optical) ISI Values for the lot of thromboplastin are given.

----------**End Of Report**----------

Bijan Basak

Dr. BIJAN BASAK
MD (Pathology), PDF (Oncopathology)
Consultant Oncopathologist | REGD. NO: (WBMC65329)

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THE MISSION HOSPITAL DURGAPUR | FUTURISTIC TECHNOLOGY. BRILLIANT MINDS.

ARTMENT OF LABORATORY SERVICES

Patient Name	Mr. ALOKE KUMAR MUKHERJEE	Bill No	OPCA326/173197
Source	OPD	Sample Date	03/09/2026 3:22PM
UHID	842747	Receiving Date	03/09/2026 4:24PM
Age/Gender	66 Yrs/Male	Report Date	04/09/2026 1:23AM
Bed No/Ward	OPD	Report Status	Final
Referred By	Dr. ASHISH KRISHNA	Lab No	362504047089
Sample Name	Serum	ManualDept No.	

HEPATITIS B SURFACE ANTIGEN (ECI)

HEPATITIS B SURFACE ANTIGEN (ECI)	0.13	Index	0 - 1
INTERPRETATION-	Non Reactive		

Interpretation:

Reference Group	Value
Non Reactive	<0.90
Borderline	0.90-1.00
Reactive	>1.00

The test is for the qualitative detection of Hepatitis B Surface Antigen (HBsAg) in human serum and plasma. Hepatitis B is a major public health problem of global importance. HBsAg is the first marker to appear following infection, and is the best indirect indicator of potentially infectious sera. If test result is >=0.90, the sample should be tested by supplemental tests to confirm the result. A repeatedly reactive sample confirmed by supplemental tests should be considered positive for HBsAg. In the case of repeatedly borderline results, analysis of follow up samples is recommended. HBsAg results should only be used and interpreted in the context of the overall clinical picture. A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus. Levels of HBsAg may be undetectable both in early infection and late after infection. In rare cases HBsAg tests do not detect certain HBV mutant strain. Heterophilic antibodies in serum or plasma samples may cause interference in immunoassays. Sensitivity of the VITROS HBsAg is <0.10 IU/mL. **All positive samples should be confirmed by Hepatitis B confirmation test & other Hepatitis B serological markers.**

Dr. PALASH DAS
DPH,MD(MICROBIOLOGY) Re
No. 64476(WBMC)
CONSULTANT (MICROBIOLO
INFECTION CONTROL)

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...ue the recalled report.

Printed at :21/08/2026 12:00

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Bed No/Ward	OPD	Report Status	Final
Referred By	Dr. ASHISH KRISHNA	Lab No	362504047089
Sample Name	Serum	ManualDept No.	

Investigation Name	Result	Units	Bio. Ref.Interval

SEROLOGY

ANTIBODY TO HEPATITIS C VIRUS (ECI)

ANTIBODY TO HEPATITIS C VIRUS (ECI)	0.44	Index	0 - 1
INTERPRETATION-	Non Reactive		

Interpretation:

Reference Group	Value
Non Reactive	<0.90
Borderline	0.90-1.00
Reactive	>1.00

The test is for the qualitative detection of antibodies to hepatitis C virus (anti-HCV) in human serum and plasma. The hepatitis C virus (HCV) is now known to be the causative agent for most, if not all, blood-borne non-A, non-B hepatitis (NANBH). Studies throughout the world indicate that HCV is transmitted through contaminated blood and blood products, through blood transfusions or through other close, personal contacts. The presence of anti-HCV indicates that an individual may have been infected with HCV and may be capable of transmitting HCV infection. If test result is >=0.90, the sample should be tested by supplemental tests to confirm the result. A repeatedly reactive sample, confirmed by supplemental tests should be considered positive for anti-HCV. In the case of repeatedly borderline results, analysis of follow-up samples is recommended. A negative test result does not exclude the possibility of exposure to or infection with HCV. HCV antibodies may be undetectable in some stages of the infection and in some clinical conditions. False positive results are seen in Autoimmune diseases, Rheumatoid factor, Hyper gamma globulinemia, Para proteinemia, passive antibody transfer, Anti-idiotypes & Anti superoxide dismutase.

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